AM-02 Diabetic Emergency - Adult

MEDICAL-DIRECTOR APPROVED APP CONTENT | SOURCE-PDF REPLACEMENT | Reconciled October 16, 2026

Scope: Use for symptomatic hypoglycemia, hyperglycemia/DKA-HHS concern, or an unexplained metabolic presentation. Claiborne EMS does not administer insulin under AM-02.

Glucose, airway, and disposition pathway

1. Check glucose + ABCs	Assess airway, breathing, circulation, mental status, focal deficits, seizure, and likely cause.
2. Glucose below 70?	Treat if symptomatic or symptoms cannot be reliably assessed because of altered mental status.
3. Safe swallow?	If alert and protecting airway: oral glucose 15-25 g. If unsafe/ineffective: D10 IV/IO or glucagon IM.
4. Titrate D10	D10 up to 250 mL IV/IO, titrated to improving mental status and glucose at least 70 mg/dL; maximum 25 g.
5. No access	Glucagon 1 mg IM when oral treatment is unsafe and vascular access is not promptly available.
6. Reassess	Parenteral treatment: glucose + neuro exam at 5 min. Oral treatment: repeat glucose within 10-15 min.
7. Hyperglycemia/DKA-HHS?	Consider glucose at least 250, Kussmaul breathing, vomiting, abdominal pain, dehydration, infection, shock, or SGLT2 use.
8. Transport/refusal	Transport high-risk medication exposure, recurrence, seizure, pregnancy, overdose, uncertain cause, DKA/HHS, or persistent deficit.

Key treatment and disposition

Item	Approved action
Fluid	For hypotension or poor perfusion only: isotonic crystalloid 500 mL IV/IO aliquots; 250 mL for fluid-overload risk; maximum 2 L.
Non-transport	Consider only after a known/clear cause, normal baseline mental/neuro status and capacity, glucose at least 80 and stable on repeat, food intake, and responsible-adult supervision.

Do not give oral glucose, food, fluid, or medication to a patient who cannot reliably swallow or protect the airway.

AM-02 Diabetic Emergency - Adult

SAFETY, LINKED PATHWAYS, AND RELEASE CONTROL

Safety checks

- Do not use D50 as the routine adult treatment under AM-02. Avoid insulin and do not change an insulin pump under this protocol.

- Strongly recommend transport after sulfonylurea or long-acting-insulin exposure, recurrent hypoglycemia, seizure, pregnancy, overdose, unclear cause, or absent supervision.

- Persistent focal deficits after correction remain a stroke alert; do not attribute them to glucose alone.

- Do not give routine fluid for uncomplicated hyperglycemia without hypotension, poor perfusion, dehydration, or another specific indication.

Linked pathways

Pathway	Use when
UP-04	Persistent altered mental status or metabolic/toxic cause
UP-13	Active seizure or persistent postictal findings
UP-14	Persistent focal deficit after glucose correction
AM-05	Persistent hypotension or poor perfusion

Documentation and release control

- Document focused findings, BP/rhythm/ECG when applicable, all interventions with dose/route/time/response, serial reassessments, destination decision, and notifications.

- This source PDF replaces the legacy imported sheet and matches the approved Claiborne EMS app content reconciled October 16, 2026.

- Use the linked protocol whenever the patient crosses into another clinical pathway. Follow agency release governance, training requirements, and current medication/device availability.